

Patient Demographics

Name	Patient ID	SSN	Gender Identity	Birth Date	
Zinke, Markus Alan	21811208	xxx-xx-0349	Male	10/11/80 (45 yrs)	
Address	Phone	Email			
1369 STODICK LN GARDNERVILLE NV 89410-7310	775-450-3353 (M)	—			
Reg Status	PCP	Date Last Verified	Next Review Date		
Verified Registration	Pt Requests No Assignment	07/20/26	01/16/27		

**Zinke, Markus Alan #21811208 (Acct:8209388254) (45
year old M) PCP: NO ASSIGNMENT, P****01****ED Arrival Information**

Patient Information			
Expected	Arrival	Acuity	
-	7/20/2026 13:12	Level 2	
Means of arrival	Escorted by	Service	Admission type
Private Transportation	Spouse	Internal Medicine	Emergency

Arrival complaint

headache, vision problems, "couldn't read", nausea, confusion, "jittery"

Chief Complaint

None

ED Treatment Team

Provider	Role	From	To	Phone	Pager
Sean N Kump, PA-C	Attending Provider	07/20/26 1323	07/20/26 1536	445-8772	--
Hannah Lyons, RN	Registered Nurse	07/20/26 1341	07/20/26 1609	--	--
Matthew Godin, DO	Admitting Provider	--	--	775-445-8000	--

ED Provider Notes**ED Provider Notes by Sean N Kump, PA-C**

07/20/26 1330

Author: Sean N Kump, PA-C

Service: —

Author Type: Physician Assistant

Filed: 07/20/26 1442

Date of Service: 07/20/26 1330

Status: Signed

Editor: Sean N Kump, PA-C (Physician Assistant)

Emergency Medicine Note:Patient: Markus Alan Zinke; 45 year old male^[SK.1]No chief complaint on file.^[SK.2]Mode of Arrival:^[SK.1] Private Transportation^[SK.2]**History of Present Illness**

History obtained from the patient.

This is a normally healthy 45-year-old male with no reported past medical history presents the ER today with complaints of possible TIA. Per the patient, at 10:00 a.m. this morning while at work, he developed acute onset of a headache, nausea without vomiting, and inability to read. The patient notes that he was trying to read words off of a piece of paper and was unable to do so. The patient then drove home and believes that this is secondary to low glucose levels and picked up a soda. He had no relief in his symptoms. His family noted that

ED Provider Notes (continued)**ED Provider Notes by Sean N Kump, PA-C (continued)**

07/20/26 1330

he seemed confused at that time. The patient had difficulty finding the right words to say when speaking with his family. He did not have any noticed slurred speech. The patient had no focal weakness reported. He had no noted facial droop. The patient notes that these symptoms lasted for around 15 minutes with now near complete resolution. The patient notes only persistence of a mild headache as well as mild nausea at this time. He denies any recent falls or trauma. Denies any history of CVA. Denies any chest pain, back pain, abdominal pain, urinary complaints. Denies blood thinner use.

Nursing and triage documentation reviewed.

Review of external medical records: Office visit note from 03/31/2022 was reviewed

REVIEW OF SYSTEMS: 11 Point review of systems were negative unless otherwise mentioned in HPI, or non-contributory to this visit^[SK.1]

History reviewed. No pertinent past medical history.^[SK.2]

PCP:^[SK.1] No Assignment Pt Req

No past surgical history on file.

No family history on file.^[SK.2]

Social History**Social History****Tobacco Use**

- Smoking status: Never
- Smokeless tobacco: Never

Substance Use Topics

- Alcohol use: Yes
- Drug use: Never^[SK.1]

No current outpatient medications

Outpatient medications were reviewed.^[SK.1]

No Known Allergies^[SK.2]

Physical Exam:

General: Pleasant nontoxic in no acute distress

Head: Normocephalic atraumatic

ENT: PERLA EOMI sclerae anicteric noninjected oral mucosa moist oropharynx clear

Neck: Supple no stridor or adenopathy no nuchal rigidity no JVP elevation

Lungs: Clear to auscultation bilaterally

Cardiovascular: regular rate and rhythm no obvious murmurs rubs or gallops cap refill less than 2 sec

symmetric radial pulses no pedal edema

Abdomen: good bowel sounds soft abdomen nontender nondistended negative Murphy's no McBurney's no

ED Provider Notes (continued)**ED Provider Notes by Sean N Kump, PA-C (continued)**

07/20/26 1330

peritonitis

Back: no midline tenderness to palpation or step-offs normal range of motion

Skin: intact warm dry no obvious rashes

Extremities: Able to fully range all extremities without pain, no gross abnormalities

Neuro: Cranial nerves 2-12 grossly intact. Alert oriented to person place time and situation GCS 15 symmetric face has 5/5 strength throughout normal sensation to light touch throughout no limb ataxia. No meningismus.

Patient able to read upon exam without noted difficulty, dysarthria or aphasia. NIH score of 0.

Psych: Calm and cooperative

Vital signs reviewed.^[SK.1]

Vitals

	07/20/26 1314	07/20/26 1317	07/20/26 1318
BP:			150/87
Pulse:			72
Temp:			37.1 °C (98.7 °F)
Resp:			17
Height:		190.5 cm (6' 3")	
Weight:	86.1 kg (189 lb 13.1 oz)		
SpO2:			98% ^[SK.2]

190.5 cm (6' 3")

,^[SK.1] 189 lbs 13.06 oz^[SK.2],^[SK.1] Body mass index is 23.73 kg/m².^[SK.2]

Social Determinants of Health:

NA

Results:^[SK.1]

Recent Results (from the past 48 hours)

POC Fingerstick Glucose & Heelstick

Collection Time: 07/20/26 1:26 PM

Result	Value	Ref Range
Glucose (Bedside), Whole Blood	118 (H)	70 - 110

CBC With Plt Count And Auto Diff

Collection Time: 07/20/26 1:47 PM

Result	Value	Ref Range
WBC Count	5.5	3.7 - 10.6 x10E3/uL
RBC	5.34	4.50 - 5.70 x10e6/uL
Hemoglobin	15.8	13.0 - 16.7 g/dL
Hematocrit	47.2	38.8 - 49.7 %
Mean Corpuscular Volume	88.3	83.0 - 99.0 fL
Mean Corpuscular Hemoglobin	29.6	28.0 - 33.8 pg
Mean Corpuscular HGB	33.5	33.1 - 36.5 g/dL

ED Provider Notes (continued)**ED Provider Notes by Sean N Kump, PA-C (continued)**

07/20/26 1330

Concentration

Red Cell Distribution Width	13.4	11.8 - 14.0 %
Platelet Count	251	146 - 390 x10E3/uL
Mean Platelet Volume	7.7	6.4 - 10.2 fL
Neutrophil %	65.9	41.7 - 82.3 %
Lymphocyte %	26.4	10.8 - 44.4 %
Monocyte %	5.9	5.0 - 12.8 %
Eosinophil %	0.8	0.0 - 6.6 %
Basophil %	1.0	0.0 - 1.3 %
Neutrophil #	3.60	1.40 - 8.00 x10E3/uL
Lymphocyte #	1.50	1.00 - 5.20 x10E3/uL
Monocyte #	0.30	0.16 - 1.00 x10E3/uL
Eosinophil #	0.00	0.00 - 0.80 x10E3/uL
Basophil #	0.10	0.00 - 0.30 x10E3/uL

Comprehensive Metabolic Panel

Collection Time: 07/20/26 1:47 PM

Result	Value	Ref Range
Sodium, Serum or Plasma	136	136 - 144 mmol/L
Potassium	3.7	3.6 - 5.1 mmol/L
Chloride	104	102 - 110 mmol/L
Carbon Dioxide	25	22 - 32 mmol/L
Alkaline Phosphatase	30 (L)	38 - 126 U/L
AST	22	15 - 41 U/L
ALT	29	17 - 63 U/L
BUN	17	8 - 20 mg/dL
Creatinine, Serum or Plasma	1.09	0.80 - 1.40 mg/dL
Calcium	9.6	8.7 - 10.3 mg/dL
Protein, Total, Serum or Plasma	7.4	6.4 - 8.2 g/dL
Albumin	4.7	3.5 - 4.8 g/dL
A/G Ratio	1.7	1.0 - 2.2 ratio
Glomerular Filtration Rate (GFR)	73	>60 mL/min/1.7
Globulins, Total	2.7	2.6 - 3.1 g/dL
Glucose, Serum or Plasma	104 (H)	60 - 99 mg/dL
Bilirubin, Total	0.7	0.4 - 2.0 mg/dL
Anion Gap	7	2 - 11 mmol/L

Blood Bank Hold Specimen

Collection Time: 07/20/26 1:47 PM

Result	Value	Ref Range
Blood Bank Hold Clot	BB Draw is available for 72 hours for testing.	

Prothrombin Time/International Normalized Ratio

Collection Time: 07/20/26 1:47 PM

Result	Value	Ref Range
Prothrombin Time	13.8	11.7 - 14.4 seconds

ED Provider Notes (continued)**ED Provider Notes by Sean N Kump, PA-C (continued)**

07/20/26 1330

INR	1.0	0.9 - 1.1 ratio
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Troponin I, High Sensitivity Reflex

Collection Time: 07/20/26 1:47 PM

Result	Value	Ref Range
Troponin-I, HS Baseline	<3	<19 ng/L

12 Lead ECG

Collection Time: 07/20/26 1:53 PM

Result	Value	Ref Range
Ventricular Rate	67	BPM
Atrial Rate	67	BPM
P-R Interval	162	ms
QRS Duration	124	ms
Q-T Interval	398	ms
QTC Calculation (Bezjet)	420	ms
Calculated P Axis	69	degrees
Calculated R Axis	71	degrees
Calculated T Axis	55	degrees

Diagnosis

Normal sinus rhythm

Right bundle branch block

Abnormal ECG

No previous ECGs available

Confirmed by Walther, RYanne (33) on 7/20/2026 2:24:37 PM

CTA Head W IV Cont

Result Date: 7/20/2026

1. No major arterial occlusion or significant stenosis identified. Electronically Signed by: Stephen Loos, MD
7/20/2026 2:01 PM

CTA Neck W IV Cont

Result Date: 7/20/2026

1. No major arterial occlusion or significant stenosis identified. Electronically Signed by: Stephen Loos, MD
7/20/2026 2:01 PM

CT Head WO IV Cont Stroke Protocol

Result Date: 7/20/2026

1. No acute intracranial abnormality. Electronically Signed by: John Kuipers 7/20/2026 1:39 PM^[SK.2]

EKG shows normal sinus rhythm rate of 67, QT/QTC of 390/420, no ST-elevation MI, right bundle-branch block.^[SK.3]

ED Course:^[SK.1]

Clinical Impressions as of 07/20/26 1442

TIA (transient ischemic attack) - Suspected

ED Provider Notes (continued)**ED Provider Notes by Sean N Kump, PA-C (continued)**

07/20/26 1330

Aphasia - Resolved^[SK.2]**ER Medications given^[SK.1]**

Medication Administrations (24 hours)

(last 24 hrs)

Order ID	Medication Name	Action Time	Action	Site	Route	Dose
481588051	iopamidol (ISOVUE-370) 76 % injection 75 mL	07/20/26 1352	Given		Intravenous	75 mL ^[SK.2]

Procedures:

Procedures

Differential Diagnosis:

CVA, ICH, TIA, hypoglycemia, trauma, infectious process, TGA

MEDICAL DECISION MAKING:

This is a 45-year-old male presents to the ER today for the above-mentioned complaint. Patient appears well, nontoxic in no distress. In triage, code stroke was initiated as last known normal was at 10:00 a.m. this morning, 3.5 hours prior to my exam. I initially evaluated the patient in the CT room. Initial exam is nonfocal. Patient's sitting upright on the edge of the CT scanner in no acute distress. NIH score initially is 0. Point of care glucose is 118. Blood pressure is 150/87. The patient notes near complete resolution of all symptoms at this time other than a mild headache as well as nausea. At this time, I have considered a broad differential as noted above. Immediately following my exam, I did discuss this case with University of Utah neurology. I am thankful for their consultation. Neurology recommends proceeding with TIA workup at this time to include MRI imaging. She would like the patient to receive 300 mg of Plavix, full-dose aspirin today and then continuation of 75 mg of Plavix times 21 days and lifetime use of 81 mg ASA. Neurology further recommend high-dose intensity statin with a goal LDL less than 70. At this time, patient is not currently a TNK candidate as he has had complete resolution of symptoms and with an NIH score of 0. Patient will be admitted to the hospitalist service for MRI and further evaluation and care. Laboratory studies in the ER per code stroke protocol were ordered to include CBC, CMP, troponin, EKG^[SK.1]

CT imaging of the head without contrast returned negative for acute abnormality.

CT angiography of the head and neck returned negative. I independently reviewed laboratory studies. CBC and CMP are noncontributory. PT/INR is within normal limits. Troponin is negative. The patient was admitted to the hospitalist service in stable condition. Patient and hospitalist agrees with the admission. Patient made aware that following hospital protocol, patient requires admission to hospitalist service for MRI imaging.^[SK.3]

ED Provider Notes (continued)**ED Provider Notes by Sean N Kump, PA-C (continued)**

07/20/26 1330

CRITICAL CARE:

I have personally provided 35 minutes of discontinuous critical care time seeing, evaluating, and treating this patient. This time is exclusive of time spent on separate procedures. Upon my evaluation, this patient had a high probability of imminent or life-threatening deterioration due to code stroke with consideration for TNK, which required my direct attention, intervention, and personal management. Time includes review of laboratory data, radiology results, discussion with consultants, and monitoring for potential decompensation.

Interventions were performed as documented in this note.

Mandatory Screening for Selected Patients:

Sexually transmitted disease testing was offered in the emergency department: Not applicable

Syphilis screen was offered in the emergency department: not applicable as the patient was not pregnant

New Meds / Prescriptions:^[SK.1]Patient's Medications

No medications on file^[SK.2]

Diagnoses:^[SK.1]**Final diagnoses:**

TIA (transient ischemic attack) - Suspected

Aphasia - Resolved^[SK.2]

Disposition:^[SK.1]

Admit

SEAN N KUMP, PA-C^[SK.2]

Emergency Medicine

This document was created using a computerized dictation system. Although I have reviewed this document, there may still be some phonetic and other typographical errors due to imperfections in the software program and should not be construed in any way to compromise the patient's medical care.

During evening hours, CT/MRI studies are interpreted by a remote teleradiology service. Per hospital protocol, Xray images are read by an attending physician. All studies are over-read by in house radiology in the morning and the patient will be contacted by the hospital for any significant over-reads.

Sean N Kump, PA-C

07/20/26 1442

^[SK.2]

Attribution Key

ED Provider Notes (continued)**ED Provider Notes by Sean N Kump, PA-C (continued)**

07/20/26 1330

SK.1 - Sean N Kump, PA-C on 07/20/26 1330

SK.2 - Sean N Kump, PA-C on 07/20/26 1442

SK.3 - Sean N Kump, PA-C on 07/20/26 1439

ED Course

There is no data filed.

Dictations

None

ED Diagnoses

Diagnosis	Description	Comment	Associated Orders
Final diagnoses			
TIA (transient ischemic attack)	TIA (transient ischemic attack)	Suspected	AMB REFERRAL TO NEUROLOGY
Aphasia	Aphasia	Resolved	--

ED Disposition

ED Disposition	Condition	Comment
Admit	--	Level of Care: Acute [13] Supervision: No Infection Control: No

Results

Procedure	Component	Value	Ref Range	Date/Time
12 Lead ECG [481579032]				Collected: 07/20/26 1353
Order Status: Completed				Updated: 07/20/26 1424
	Ventricular Rate	67	BPM	
	Atrial Rate	67	BPM	
	P-R Interval	162	ms	
	QRS Duration	124	ms	
	Q-T Interval	398	ms	
	QTC Calculation (Bezot)	420	ms	
	Calculated P Axis	69	degrees	
	Calculated R Axis	71	degrees	
	Calculated T Axis	55	degrees	
	Diagnosis	--		
		Normal sinus rhythm		
		Right bundle branch block		
		Abnormal ECG		
		No previous ECGs available		
		Confirmed by Walther, Ryanne (33) on 7/20/2026 2:24:37 PM		
POC Fingerstick Glucose & Heelstick [481583491] (Abnormal)				Collected: 07/20/26 1326
Order Status: Completed				Updated: 07/20/26 1336
	Glucose (Bedside), Whole Blood	118 ^	70 - 110	

Imaging Results

Transthoracic Echo (TTE), Complete (Edited Result - FINAL)

Result time 07/21/26 15:25:12

Edited Result - FINAL by David M Baker, MD (07/21/26 15:25:12)**Narrative:**

Version 2

1600 Medical Parkway
 Carson City, NV 89706
 Phone (775) 445-8846
 Fax (775) 888-3204

Echocardiogram Report

Name: ZINKE, MARKUS ALAN Study Date: 07/21/2026 Height: 75 in
 MRN: 21811208 Patient Location: RMCCPU 63 CDU 01 CTH Weight: 188 lb
 DOB: 10/11/1980 Gender: Male BSA: 2.1 m2
 Age: 45 yrs Account #: 378162649 BP: 121/70 mmHg
 Reason For Study: TIA/ CVA HR: 64
 Ordering Physician: REAGAN, TIMOTHY Performed By: Jade Watson, RDCS

Study Comments: Prior Exam No. Two forms of patient identification verified. A two-dimensional transthoracic echocardiogram with color flow and Doppler was performed. The procedure was explained to the patient and/or family member present. Digital images were acquired using a GE s70 Ultrasound System. The study was done portably. The study was technically adequate. The patient was in normal sinus rhythm during the exam.

Left Ventricle: The left ventricle is normal in size. The Ejection Fraction estimate is 55-60%.

Left ventricular systolic function is normal. No regional wall motion abnormalities noted. A variety of Doppler measurements indicate normal left ventricular diastolic function.

Right Ventricle: The right ventricle is normal in size and function.

Atria: The left atrial volume index is estimated to be 34-41 ml/m2. The left atrial size is normal. The right atrium is normal in size. Injection of contrast documented no interatrial shunt.

Mitral Valve: The mitral valve leaflets are sclerotic, but show no functional abnormalities. There is no mitral valve stenosis. The Mitral valve area is calculated at 4.3 cm² by the VTI. The mean pressure gradient is 0.84 mmHg. There is trace mitral regurgitation.

Aortic Valve: The aortic valve is trileaflet. The aortic valve opens well. There is aortic valvular sclerosis with no stenosis or regurgitation. The aortic valve area is calculated at 4.1 cm² by the continuity equation. The mean pressure gradient is 3.3 mmHg.

Tricuspid Valve: The tricuspid valve leaflets are thin and pliable. There is a trace or physiologic amount of tricuspid regurgitation.

Pulmonic Valve: The pulmonic valve is not well seen, but is grossly normal. There is no pulmonic valvular regurgitation.

Great Vessels: The aortic root is normal size. The abdominal aorta measures 19 mm in size. The IVC was normal in size at < 23 mm and collapsed >50% with respiration (RAP 3 mmHg).

Pericardium/Pleural: There is no pericardial effusion.

MMode/2D Measurements & Calculations

LVIDd: 5.0 cm FS: 23.7 % LV mass(C)d: 144.0 grams Ao root diam: 3.5 cm
 LVIDs: 3.8 cm LA Volume Index: 24.5 ml/m2
 LVPWd: 0.92 cm
 IVSd: 0.77 cm

LVOT diam: 2.2 cm EDV(MOD-sp4): 90.4 ml EDV(MOD-sp2): 122.2 ml CO(MOD-sp4): 3.0 l/min
 ESV(MOD-sp4): 43.4 ml ESV(MOD-sp2): 41.8 ml
 EF(MOD-sp4): 51.9 % EF(MOD-sp2): 65.8 %

LA ESV-A/L: 52.4 ml RA Length: 4.6 cm LAV(MOD-sp2): 56.6 ml LAV(MOD-sp4): 37.3 ml
 RA Width: 3.6 cm

Imaging Results (continued)

LV EF (MOD) bp: 58.7 % TAPSE: 1.6 cm

Time Measurements

Aortic R-R: 0.94 sec

Aortic HR: 63.5 BPM

Doppler Measurements & Calculations

MV E max vel: 63.5 cm/sec E/E' sept: 6.1 MV dec slope: 248.6 cm/sec² Ao V2 max: 122.4 cm/sec

Med Peak E' Vel: 10.4 cm/sec E/E' lat: 4.4 MV dec time: 0.26 sec Ao max PG: 6.0 mmHg

Lat Peak E' Vel: 14.3 cm/sec MV V2 max: 78.8 cm/sec Ao mean PG: 3.3 mmHg

MV A max vel: 38.6 cm/sec MV max PG: 2.5 mmHg Ao V2 VTI: 24.1 cm

MV E/A: 1.6 MV mean PG: 0.84 mmHg AVA(I,A): 4.1 cm²

MVA(VTI): 4.3 cm²

LV V1 max: 120.0 cm/sec CO(Ao): 14.8 l/min PA acc time: 0.12 sec RAP systole: 3.0 mmHg

LV V1 max PG: 5.8 mmHg CI(Ao): 6.9 l/min/m²

LV V1 mean PG: 2.9 mmHg CO(LVOT): 6.3 l/min

LV V1 VTI: 25.3 cm

CI(LVOT): 2.9 l/min/m²

PA pr(Accel): 25.1 mmHg E/E' AVG: 5.3

Interpretation Summary

Left ventricular systolic function is normal. The Ejection Fraction estimate is 55-60%

No significant valvular abnormalities

Injection of contrast documented no interatrial shunt.

Electronically signed by: David Baker, M.D. on 07/21/2026 03:25 PM

MRI Brain WO IV Cont (Final result)

Result time 07/20/26 16:59:46

Final result by Ryan Joseph Redelman, MD (07/20/26 16:59:46)**Impression:**

1. Left cerebellar cavernoma and DVA.

Electronically Signed by: Ryan Redelman, MD 7/20/2026 4:59 PM

Narrative:

EXAM: MRI BRAIN WO IV CONT

INDICATION: 45 years old, Male. Transient ischemic attack (TIA)

TECHNIQUE: Routine MRI of the brain without contrast. No adverse affect.

CONTRAST: None

COMPARISON: None.

FINDINGS:

Imaging Results (continued)

Brain: No diffusion restriction to suggest acute infarct. Major intracranial flow voids are preserved. Ventricles and sulci are unremarkable. White matter unremarkable. No intracranial mass identified. No fluid collection identified.
Orbits: Visualized orbits are unremarkable.
Sinuses: Visualized paranasal sinuses grossly unremarkable.
Mastoid air cells: Mastoid air cells are clear.
Marrow: Bone marrow grossly unremarkable.
Other: Left cerebellar cavernoma and DVA.

CTA Head W IV Cont (Final result)

Result time 07/20/26 14:01:51

Final result by Stephen Loos, MD (07/20/26 14:01:51)**Impression:**

1. No major arterial occlusion or significant stenosis identified.

Electronically Signed by: Stephen Loos, MD 7/20/2026 2:01 PM

Narrative:

EXAMINATION: CTA HEAD W IV CONT, CTA NECK W IV CONT

INDICATION: 45 years old, Male. Neuro deficit, acute, stroke suspected

COMPARISON: No relevant studies available.

TECHNIQUE: Axial contrast-enhanced imaging of the head and neck was performed following the uneventful administration of intravenous contrast. Sagittal, coronal, and 3-D reformats were then rendered. All CT scans at this facility use dose modulation, iterative reconstruction, and/or size-based dosing when appropriate to reduce radiation dose to as low as reasonably achievable. Contrast was administered per approved written protocol.

3-D multiplanar MIP images re-created separately on an independent workstation and are reviewed. Carotid stenosis was calculated using NASCET criteria adapted for this modality.

The patient has had one other CT scans within the last 12 months.

CONTRAST: 138mL Isovue 370.

FINDINGS:

Partially visualized lungs: Clear.

Thoracic aorta: No visualized aneurysm or dissection.

Partially visualized great vessels: No significant stenosis.

Neck soft tissues: Unremarkable.

Right side.

Common carotid artery: No significant stenosis or evidence for dissection.

Internal carotid artery: No significant stenosis or evidence for dissection.

Vertebral artery: No significant stenosis or evidence for dissection.

Left side.

Common carotid artery: No significant stenosis or evidence for dissection.

Imaging Results (continued)

Internal carotid artery: No significant stenosis or evidence for dissection.
Vertebral artery: No significant stenosis or evidence for dissection.

Intracranial arteries:

Internal carotid arteries: No significant stenosis. No aneurysm identified.
Anterior cerebral arteries: No significant stenosis. No aneurysm identified.
Middle cerebral arteries: No significant stenosis. No aneurysm identified.
Posterior cerebral arteries: No significant stenosis. No aneurysm identified.
Basilar artery: No significant stenosis. No aneurysm identified.

Other: No significant other intracranial abnormality identified.

CTA Neck W IV Cont (Final result)

Result time 07/20/26 14:01:51

Final result by Stephen Loos, MD (07/20/26 14:01:51)**Impression:**

1. No major arterial occlusion or significant stenosis identified.

Electronically Signed by: Stephen Loos, MD 7/20/2026 2:01 PM

Narrative:

EXAMINATION: CTA HEAD W IV CONT, CTA NECK W IV CONT

INDICATION: 45 years old, Male. Neuro deficit, acute, stroke suspected

COMPARISON: No relevant studies available.

TECHNIQUE: Axial contrast-enhanced imaging of the head and neck was performed following the uneventful administration of intravenous contrast. Sagittal, coronal, and 3-D reformats were then rendered. All CT scans at this facility use dose modulation, iterative reconstruction, and/or size-based dosing when appropriate to reduce radiation dose to as low as reasonably achievable. Contrast was administered per approved written protocol.

3-D multiplanar MIP images re-created separately on an independent workstation and are reviewed. Carotid stenosis was calculated using NASCET criteria adapted for this modality.

The patient has had one other CT scans within the last 12 months.

CONTRAST: 138mL Isovue 370.

FINDINGS:

Partially visualized lungs: Clear.

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Partially visualized great vessels: No significant stenosis.

Neck soft tissues: Unremarkable.

Right side.

Common carotid artery: No significant stenosis or evidence for dissection.

Internal carotid artery: No significant stenosis or evidence for dissection.

Vertebral artery: No significant stenosis or evidence for dissection.

Left side.

Common carotid artery: No significant stenosis or evidence for dissection.

Internal carotid artery: No significant stenosis or evidence for dissection.

Vertebral artery: No significant stenosis or evidence for dissection.

Intracranial arteries:

Internal carotid arteries: No significant stenosis. No aneurysm identified.

Anterior cerebral arteries: No significant stenosis. No aneurysm identified.

Middle cerebral arteries: No significant stenosis. No aneurysm identified.

Posterior cerebral arteries: No significant stenosis. No aneurysm identified.

Basilar artery: No significant stenosis. No aneurysm identified.

Other: No significant other intracranial abnormality identified.

CT Head WO IV Cont Stroke Protocol (Final result)

Result time 07/20/26 13:39:52

Final result by John A Kuipers, DO (07/20/26 13:39:52)

Impression:

1. No acute intracranial abnormality.

Electronically Signed by: John Kuipers 7/20/2026 1:39 PM

Narrative:

Exam: CT HEAD WO IV CONT STROKE PROTOCOL

Indication: 45 years old, Male. Neuro deficit, acute, stroke suspected
CVA

Comparison: None.

CONTRAST: None

Technique: Axial CT images obtained with reformats provided and reviewed. All CT scans at this facility use dose modulation, iterative reconstruction, and/or size-based dosing when appropriate to reduce radiation dose to as low as reasonably achievable.

The patient has had 0 other CT scans within the last 12 months.

FINDINGS:

Brain: No evidence of acute intracranial hemorrhage. No definite CT evidence of acute territorial infarct. No intracranial mass or mass effect. No midline shift. White matter unremarkable. Borderline tonsillar ectopia.

Ventricles and sulci: Normal. Patent basal cisterns.

Soft tissues: Soft tissues unremarkable.

Bones: Unremarkable

Sinuses and mastoid air cells: Visualized sinuses are well aerated.

Please note CT is not sensitive for the detection of acute infarct and recommend MRI as clinically indicated.

ECG Results

12 Lead ECG (Edited Result - FINAL)

Collection Time	Result Time	Ventricular Rate	Atrial Rate	P-R Interval	QRS Duration	Q-T Interval	QTC Calculation (Beze t)	Calculated P Axis	Calculated R Axis	Calculated T Axis	Diagnosis
07/20/26 13:53:50	07/20/26 14:24:39	67	67	162	124	398	420	69	71	55	Normal sinus rhythm Right bundle branch block Abnormal ECG No previous ECGs available Confirmed by Walth er, Ryan ne (33) on 7/20/2026 2:24:37 PM

Edited Result - FINAL

Code Onset/Outcome

Date and Time	Time Code Team Page	Code Location/Unit	Event Type	Witnessed	Conscious at Onset	Pulse Present at Onset	Respirations Present at Onset	Ventilated Pre-Arrest	Telemetr y/Car diac Moni tor	Cardiac Rhyt hm	Ectopy	Ectopy Frequency	Type of Ventilation	User
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Code Onset/Outcome (continued)

Date and Time	Time Code	Team Page	Code Location/Unit	Event Type	Witnessed	Conscious at Onset	Pulse Present at Onset	Respirations Present at Onset	Ventilated Pre-Arrest	Telemetry/Cardiac Monitor	Cardiac Rhythm	Ectopy	Ectopy Frequency	Type of Ventilation	User
07/21/26 0730	--	--	--	--	--	--	--	--	--	--	NSR	--	--	--	CL
07/20/26 1914	--	--	--	--	--	--	--	--	--	--	NSR	--	--	--	BC
07/20/26 1721	--	--	--	--	--	--	--	--	--	--	NSR	--	--	--	CL

Home Medications

No medications on file.

Medication Administration from 07/20/2026 1312 to 07/20/2026 1609

Date/Time	Order	Dose	Route	Action	Action by	Comments
07/20/2026 1352 PDT	iopamidol (ISOVUE-370) 76 % injection 75 mL	75 mL	Intravenous	Given	Kiley Cook	--
07/20/2026 1500 PDT	metoclopramide (REGLAN) injection 10 mg 2 mL	10 mg	Intravenous	Given	Hannah Lyons, RN	--
07/20/2026 1500 PDT	diphenhydrAMINE (BENADRYL) 50 mg/mL injection 25 mg	25 mg	Intravenous	Given	Hannah Lyons, RN	--
07/20/2026 1459 PDT	clopidogrel (PLAVIX) tablet 300 mg	300 mg	Oral	Given	Hannah Lyons, RN	--
07/20/2026 1459 PDT	aspirin chewable tablet 324 mg	324 mg	Chew	Given	Hannah Lyons, RN	--
07/20/2026 1545 PDT	docusate sodium (COLACE) capsule 100 mg	100 mg	Oral	Not Given	Christina Lepore, RN	--

ED Prescriptions

Medication	Sig	Dispense	Start Date	End Date	Auth. Provider
aspirin 81 mg EC tablet	Take 1 tablet (81 mg) by mouth once daily.	--	7/22/2026	--	Timothy D Reagan, NP
clopidogrel (PLAVIX) 75 mg tablet	Take 1 tablet (75 mg) by mouth once daily for 20 days.	20 tablet	7/22/2026	8/11/2026	Timothy D Reagan, NP
atorvastatin (LIPITOR) 20 mg tablet	Take 1 tablet (20 mg) by mouth daily at 9 pm.	30 tablet	7/21/2026	--	Timothy D Reagan, NP

ED Medication Orders (From admission, onward)

Start	Ordered	Status	Ordering Provider
07/21/26 2100	07/21/26 1100 atorvastatin (LIPITOR) tablet 20-mg Daily at 2100 Route: Oral Ordered Dose: 20 mg	Discontinued	REAGAN, TIMOTHY D
07/21/26 0900	07/20/26 1541 aspirin enteric-coated tablet 81-mg Daily Route: Oral Ordered Dose: 81	Discontinued	REAGAN, TIMOTHY D

ED Medication Orders (continued) (From admission, onward)

Start	Ordered		Status	Ordering Provider
		mg		
07/21/26 0900	07/20/26 1541	clopidogrel (PLAVIX) tablet 75 mg Daily Route: Oral Ordered Dose: 75 mg	Discontinued	REAGAN, TIMOTHY D
07/20/26 1545	07/20/26 1541	docusate sodium (COLACE) capsule 100 mg Daily Route: Oral Ordered Dose: 100 mg	Discontinued	REAGAN, TIMOTHY D
07/20/26 1538	07/20/26 1541	sodium chloride 0.9 % infusion As needed Route: Intravenous Ordered Dose: 30 mL/hr	Discontinued	REAGAN, TIMOTHY D
07/20/26 1538	07/20/26 1541	polyethylene glycol 3350 (MIRALAX) packet 17 gram Daily as needed Route: Oral Ordered Dose: 17 gram	Discontinued	REAGAN, TIMOTHY D
07/20/26 1538	07/20/26 1541	senna (SENNAKOT) 8.6 mg 2-tablet Daily PRN Route: Oral Ordered Dose: 2 tablet	Discontinued	REAGAN, TIMOTHY D
07/20/26 1538	07/20/26 1541	melatonin tablet 3 mg At bedtime as needed Route: Oral Ordered Dose: 3 mg	Discontinued	REAGAN, TIMOTHY D
07/20/26 1538	07/20/26 1541	ondansetron ODT-dispersable tablet 4 mg Every 8 hours as needed Route: Oral Ordered Dose: 4 mg	Discontinued	REAGAN, TIMOTHY D
		Placed in "Or" Linked Group		
07/20/26 1538	07/20/26 1541	ondansetron (ZOFTRAN) injection 4 mg Every 8 hours as needed Route: Intravenous Ordered Dose: 4 mg	Discontinued	REAGAN, TIMOTHY D
		Placed in "Or" Linked Group		
07/20/26 1538	07/20/26 1541	promethazine (PHENERGAN) tablet 12.5 mg Every 6 hours as needed Route: Oral Ordered Dose: 12.5 mg	Discontinued	REAGAN, TIMOTHY D
07/20/26 1538	07/20/26 1541	prochlorperazine (COMPAZINE) tablet 5 mg Every 6 hours as needed	Discontinued	REAGAN, TIMOTHY D

ED Medication Orders (continued) (From admission, onward)

Start	Ordered		Status	Ordering Provider
		Route: Oral Ordered Dose: 5 mg		
		Placed in "Or" Linked Group		
07/20/26 1538	07/20/26 1541	prochlorperazine (COMPAZINE) injection 5 mg Every 6 hours as needed Route: Intravenous Ordered Dose: 5 mg	Discontinued	REAGAN, TIMOTHY D
		Placed in "Or" Linked Group		
07/20/26 1538	07/20/26 1541	diphenhydramine (BENADRYL) capsule 25 mg 3 times daily prn Route: Oral Ordered Dose: 25 mg	Discontinued	REAGAN, TIMOTHY D
		Placed in "Or" Linked Group		
07/20/26 1538	07/20/26 1541	diphenhydramine (BENADRYL) 50 mg/mL injection 25 mg 3 times daily prn Route: Intravenous Ordered Dose: 25 mg	Discontinued	REAGAN, TIMOTHY D
		Placed in "Or" Linked Group		
07/20/26 1538	07/20/26 1541	acetaminophen (TYLENOL) tablet 650 mg Every 6 hours as needed Route: Oral Ordered Dose: 650 mg	Discontinued	REAGAN, TIMOTHY D
		Placed in "Or" Linked Group		
07/20/26 1538	07/20/26 1541	naloxone (NARCAN) injection 0.1 mg every 2 minutes as needed Route: Intravenous Ordered Dose: 0.1 mg	Discontinued	REAGAN, TIMOTHY D
07/20/26 1500	07/20/26 1441	metoclopramide (REGLAN) injection 10 mg 2 mL Once Route: Intravenous Ordered Dose: 10 mg	Last MAR action: Given	KUMP, SEAN N
07/20/26 1500	07/20/26 1441	diphenhydramine (BENADRYL) 50 mg/mL injection 25 mg Once Route: Intravenous Ordered Dose: 25 mg	Last MAR action: Given	KUMP, SEAN N
07/20/26 1500	07/20/26 1441	clopidogrel (PLAVIX) tablet 300 mg Once Route: Oral Ordered Dose: 300 mg	Last MAR action: Given	KUMP, SEAN N
07/20/26 1500	07/20/26 1441	aspirin chewable tablet 324 mg Once	Last MAR action: Given	KUMP, SEAN N

ED Medication Orders (continued) (From admission, onward)

Start	Ordered		Status	Ordering Provider
		Route: Chew Ordered Dose: 324 mg		
07/20/26 1351	07/20/26 1352	iopamidol (ISOVUE-370) 76 % injection 75 mL Once in imaging Route: Intravenous Ordered Dose: 75 mL	Last MAR action: Given	KUMP, SEAN N

Code, Iso, Restraint (From admission, onward)

Start	Ordered		Status	Ordering Provider
07/20/26 1539	07/20/26 1541	Code Status - Full Code Continuous	Acknowledged	REAGAN, TIMOTHY D

ED Imaging Orders (From admission, onward)

Start	Ordered		Status	Ordering Provider
07/20/26 1541	07/20/26 1541	MRI Brain WO IV Cont 1 time imaging	Final result	REAGAN, TIMOTHY D
07/20/26 1321	07/20/26 1321	CT Head WO IV Cont Stroke Protocol 1 time imaging	Final result	WALTHER, RYANNE
07/20/26 1321	07/20/26 1321	CTA Head W IV Cont 1 time imaging	Final result	WALTHER, RYANNE
07/20/26 1321	07/20/26 1321	CTA Neck W IV Cont 1 time imaging	Final result	WALTHER, RYANNE

ED Micro, Lab, POCT (From admission, onward)

Start	Ordered		Status	Ordering Provider
07/21/26 0300	07/20/26 1541	Lipid Panel AM Draw - Lab	Final result	REAGAN, TIMOTHY D
07/20/26 1412	07/20/26 1411	Abo Rh Confirmation Once - Routine - Lab	Canceled	KUMP, SEAN N
07/20/26 1321	07/20/26 1321	CBC With Plt Count And Auto Diff STAT Collect/Perform - Lab	Final result	WALTHER, RYANNE
07/20/26 1321	07/20/26 1321	Comprehensive Metabolic Panel STAT Collect/Perform - Lab	Final result	WALTHER, RYANNE
07/20/26 1321	07/20/26 1321	Blood Bank Hold Specimen Once - Routine - Lab	Final result	WALTHER, RYANNE
07/20/26 1321	07/20/26 1321	Prothrombin Time/International Normalized Ratio STAT Collect/Perform - Lab	Final result	WALTHER, RYANNE
07/20/26 1321	07/20/26 1321	Troponin I, High Sensitivity Reflex STAT Collect/Perform - Lab	Final result	WALTHER, RYANNE

ED All Orders (From admission, onward)

Start	Ordered		Status	Ordering Provider
07/21/26 0300	07/20/26 1541	Lipid Panel AM Draw - Lab	Final result	REAGAN, TIMOTHY D
07/21/26 0000	07/21/26 1421	AMB Referral to Neurology Comments: My Clinical Question (please include follow up instructions, if any):	Ordered	REAGAN, TIMOTHY D

Start	Ordered	Status	Ordering Provider
		Cerebellar cavernoma Specialty: NEUROLOGY Provider: (Not yet assigned)	
07/20/26 1541	07/20/26 1541	PT Eval and Treat Once Completed	REAGAN, TIMOTHY D
07/20/26 1541	07/20/26 1541	MRI Brain WO IV Cont 1 time imaging Final result	REAGAN, TIMOTHY D
07/20/26 1540	07/20/26 1541	Cardiac Diet Continuous Canceled	REAGAN, TIMOTHY D
07/20/26 1540	07/20/26 1541	Transthoracic Echo (TTE), Complete Once Edited Result - FINAL	REAGAN, TIMOTHY D
07/20/26 1539	07/20/26 1541	Neuro Checks Every 4 hours Canceled	REAGAN, TIMOTHY D
07/20/26 1539	07/20/26 1541	Communication Order MD to Nursing - Order Blood Culture x2 for fever greater than 100.4 and no blood cultures in last 24 hours. Continuous Comments: Order Blood Culture x2 for fever greater than 100.4 and no blood cultures in last 24 hours. Obtain from two different sites.	REAGAN, TIMOTHY D
07/20/26 1539	07/20/26 1541	Notify Provider - No Bowel Movement Continuous Comments: If patient has not had a bowel movement for 2 consecutive days, with or without laxatives.	REAGAN, TIMOTHY D
07/20/26 1539	07/20/26 1541	Nasal Sanitizing Protocol Continuous Comments: Order Nasal Sanitizer Protocol [NUR771] if indicated by history of infection or colonization with MRSA.	REAGAN, TIMOTHY D
07/20/26 1539	07/20/26 1541	Vital Signs - Continuous - Per Unit Protocol Continuous Comments: Per Unit Protocol.	REAGAN, TIMOTHY D
07/20/26 1539	07/20/26 1541	Telemetry Monitoring Telemetry Box Continuous X 48 hours Completed	REAGAN, TIMOTHY D
07/20/26 1539	07/20/26 1541	Peripheral IV Continuous Canceled	REAGAN, TIMOTHY D
07/20/26 1539	07/20/26 1541	Up with Assistance Continuous Canceled	REAGAN, TIMOTHY D
07/20/26 1539	07/20/26 1541	Authority is delegated to the clinical RD to modify/initiate nutrition related orders as needed per protocol Continuous Canceled	REAGAN, TIMOTHY D
07/20/26 1539	07/20/26 1541	Notify Provider - If pain remains uncontrolled Continuous Comments: If patient on oral	REAGAN, TIMOTHY D

Start	Ordered		Status	Ordering Provider
		with IV breakthrough; notify provider if thirty minutes after repeat IV escalation dose, pain remains uncontrolled.		
07/20/26 1539	07/20/26 1541	Communication Order Provider to Nursing: Use the numeric rating scale (NRS) when patients can reliably report pain and Critical Care Pain Observation Tool (CPOT) in ED/ICU for patients who cannot reliably report pain. Continuous	Canceled	REAGAN, TIMOTHY D
07/20/26 1539	07/20/26 1541	Communication Order Provider to Nursing: Pulse Oximetry Per Policy. Continuous Comments: Per policy for patients on opiate pain management.	Canceled	REAGAN, TIMOTHY D
07/20/26 1539	07/20/26 1541	Communication Order Provider to Nursing: If patient is difficult to arouse and/or has respiratory depression (respiratory rate of 10 or less), stop any opiates and call a Rapid Response. Continuous	Canceled	REAGAN, TIMOTHY D
07/20/26 1539	07/20/26 1541	Low Risk of DVT Once	Completed	REAGAN, TIMOTHY D
07/20/26 1412	07/20/26 1411	Abo Rh Confirmation Once - Routine - Lab	Canceled	KUMP, SEAN N
07/20/26 1337	07/20/26 1326	POC Fingerstick Glucose & Heelstick PROCEDURE ONCE Comments: Temp: 37.1 °C (98.7 °F) Allergies on File: Patient has no known allergies.	Final result	KUMP, SEAN N
07/20/26 1321	07/20/26 1321	Communication Order MD to Nursing - Undress Patient Completely Continuous Comments: Undress Patient Completely	Canceled	REAGAN, TIMOTHY D
07/20/26 1321	07/20/26 1321	Notify Provider -- Hemodynamic Parameters Continuous	Canceled	REAGAN, TIMOTHY D
07/20/26 1321	07/20/26 1321	NIH Stroke Scale Instructions Continuous Comments: For post-thrombolytics patients: NIH Stroke Scale to be completed on transfer to a new unit, change of shift between nursing, and with any changes	Canceled	REAGAN, TIMOTHY D

Start	Ordered	Status	Ordering Provider
		in neurological status. For all other patients: NIHSS x 1 only and with any acute changes in neurological status.	
07/20/26 1321	07/20/26 1321	Vital signs Every 15 minutes Comments: Vital signs every 15 minutes until treatment decision is made, then per unit protocol.	WALTHER, RYANNE
07/20/26 1321	07/20/26 1321	Weight Once	REAGAN, TIMOTHY D
07/20/26 1321	07/20/26 1321	Bedside Swallow Evaluation - Nurse Once Comments: If patient fails swallow screening: keep patient NPO and order SLP evaluation of dysphagia. If patient passes swallow screening: reorder previous diet.	WALTHER, RYANNE
07/20/26 1321	07/20/26 1321	Neuro Checks Every 15 minutes Comments: Neuro-checks every 15 minutes until treatment decision is made, then per unit protocol.	WALTHER, RYANNE
07/20/26 1321	07/20/26 1321	Peripheral IV Continuous	WALTHER, RYANNE
07/20/26 1321	07/20/26 1321	Cardiac Monitor - Continuous Continuous	REAGAN, TIMOTHY D
07/20/26 1321	07/20/26 1321	Pulse Oximetry - Nursing - Continuous Continuous Comments: Maintain SpO2 levels between 90% - 95%. Titrate as tolerated the liter flow or FiO2 to maintain the prescribed SpO2 levels.	WALTHER, RYANNE
07/20/26 1321	07/20/26 1321	Oxygen Therapy - Nursing Continuous Comments: Maintain SpO2 levels between 90% and 95%. Titrate as tolerated the liter flow or FiO2 to maintain the prescribed SpO2 levels.	WALTHER, RYANNE
07/20/26 1321	07/20/26 1321	Depression Severity Rating Scale PHQ-9 Once Comments: Perform depression screening (PHQ-2/PHQ-9). Add follow-up with PCP to the discharge planning if positive.	REAGAN, TIMOTHY D
07/20/26 1321	07/20/26 1321	POC Fingerstick Glucose & Heelstick Request STAT Collect/Routine Perform - Lab	WALTHER, RYANNE
07/20/26 1321	07/20/26 1321	NPO Continuous	WALTHER, RYANNE
07/20/26 1321	07/20/26 1321	CBC With Plt Count And Auto Diff STAT Collect/Perform -	

Start	Ordered		Status	Ordering Provider
		Lab	Final result	WALTHER, RYANNE
07/20/26 1321	07/20/26 1321	Comprehensive Metabolic Panel STAT Collect/Perform - Lab	Final result	WALTHER, RYANNE
07/20/26 1321	07/20/26 1321	Blood Bank Hold Specimen Once - Routine - Lab	Final result	WALTHER, RYANNE
07/20/26 1321	07/20/26 1321	Prothrombin Time/International Normalized Ratio STAT Collect/Perform - Lab	Final result	WALTHER, RYANNE
07/20/26 1321	07/20/26 1321	Troponin I, High Sensitivity Reflex STAT Collect/Perform - Lab	Final result	WALTHER, RYANNE
07/20/26 1321	07/20/26 1321	CT Head WO IV Cont Stroke Protocol 1 time imaging	Final result	WALTHER, RYANNE
07/20/26 1321	07/20/26 1321	CTA Head W IV Cont 1 time imaging	Final result	WALTHER, RYANNE
07/20/26 1321	07/20/26 1321	CTA Neck W IV Cont 1 time imaging	Final result	WALTHER, RYANNE

Discharge Orders (From admission, onward)

None

Allergies (Reviewed on: 07/22/26)

No Known Allergies

Tetanus Up To Date

None

Medical History

No past medical history on file.

Surgical History

No past surgical history on file.

Social History

Tobacco History

Smoking Status

Never

Smokeless Tobacco Use

Never

Alcohol History

Alcohol Use Status

Yes

Drug Use

Drug Use Status

Never

Sexual Activity

Sexually Active

Not Asked

Other Factors

Not Asked

Social Documentation

No social documentation on file.

Family Medical History

None

ED LDA Documentation (7/19/2026 00:00 to 7/20/2026 16:09)

7/20/2026	Event	Details	User
13:47	Peripheral IV 07/20/26 Right Antecubital 18 Gauge Placed	Final Assessment Date/Final Assessment Time: 07/21/26 1535 Placement/First Assessed Date/Placement/First Assessed Time: 07/20/26 1347 Laterality: Right Location: Antecubital Size (Gauge): 18 Gauge	Emma Downs, RN
13:47:26	Peripheral IV 07/20/26 Right Antecubital 18 Gauge Assessment	Line Status: WDL; Flushed; Normal Saline Locked Site Assessment: WDL Blood Drawn and Sent to Lab: Yes Dressing Change Due Date: 07/27/26 Dressing Change: 07/20/26 Dressing Type: Transparent Dressing Status: WDL Dressing Intervention: New Dressing	Emma Downs, RN
15:59	Peripheral IV 07/20/26 Right Antecubital 18 Gauge Assessment	Line Status: WDL; Capped; Flushed; Normal Saline Locked Site Assessment: WDL Dressing Type: Transparent Dressing Status: WDL	Megan Penneau, RN

ED Events

Date/Time	Event	User	Comments
07/20/26 1312	Patient arrived in ED	VALDEZ, NADIA	--
07/20/26 1312	Emergency encounter created	VALDEZ, NADIA	--
07/20/26 1317	Triage Completed	MONTALVO, RICHARD	--
07/20/26 1321	Patient roomed in ED	KELLY, KATHERINE	To room 016
07/20/26 1323	Assign Attending	KUMP, SEAN	Sean N Kump, PA-C assigned as Attending
07/20/26 1323	Assign Physician	KUMP, SEAN	--
07/20/26 1338	Registration Started	LOBATO, DIANA	--
07/20/26 1341	Assign Nurse	LYONS, HANNAH	Hannah Lyons, RN assigned as Registered Nurse
07/20/26 1422	Registration Completed	LOBATO, DIANA	--
07/20/26 1439	Bed Requested	KUMP, SEAN	Requested: Internal Medicine
07/20/26 1536	Remove Attending	REAGAN, TIMOTHY	Sean N Kump, PA-C removed as Attending
07/20/26 1536	Assign Attending	REAGAN, TIMOTHY	Timothy D Reagan, NP assigned as Attending
07/20/26 1603	Patient transferred	LYONS, HANNAH	--
07/20/26 1609	ED Tracking End	LEPORE, CHRISTINA	--
07/20/26 1609	Patient admitted	LEPORE, CHRISTINA	--
07/20/26 1609	ED Boarder Patient	KUMP, SEAN	--

Follow-up Information

None

Discharge Instructions

None

Discharge References/Attachments**Aphasia, What Is (English)****Aspirin (English)**

Discharge References/Attachments (continued)**Atorvastatin (English)****Clopidogrel (English)****AVS Reports**

Date/Time	Report	Action	User
7/21/2026 3:46 PM	Patient Discharge Instructions	Printed	Christina Lepore, RN

Communication Routing History

None

ED Charges

Description	Code	Date	Service Prov	Modifiers	Qty	Status
HB TELEMETRY OBSERVATION	1000150	07/21/2026			16	Filed
HB WILLOW REV 250 CARE 637 GY	5040066	07/21/2026	Reagan, Timothy D		1	Filed
HB WILLOW REV 250 CARE 637 GY	5040066	07/21/2026	Reagan, Timothy D		1	Filed
HB PT EVALUATION LOW COMPLEX	7005491	07/21/2026	Vicente, Benjamin	GP	1	Filed
HB LIPID PANEL	3100153	07/21/2026	Reagan, Timothy D		1	Filed
HB VENIPUNCTURE (NON ARUP)	3000002	07/21/2026	Reagan, Timothy D		1	Filed
HB EMERGENCY DEPT VISIT 5	6000010	07/20/2026	Kump, Sean N	25	1	Filed
HB TELEMETRY OBSERVATION	1000150	07/20/2026			8	Filed
HB WILLOW REV 250 CARE 637 GY	5040066	07/20/2026	Kump, Sean N		1	Filed
HB WILLOW REV 250 CARE 637 GY	5040066	07/20/2026	Kump, Sean N		1	Filed
HB WILLOW REV 636	5040098	07/20/2026	Kump, Sean N		1	Filed
HB WILLOW REV 636	5040098	07/20/2026	Kump, Sean N		1	Filed
HB TROPONIN I	3101808	07/20/2026	Walther, Ryanne		1	Filed
HB COMPREHENSIVE METABOLIC PANEL	3100145	07/20/2026	Walther, Ryanne		1	Filed
HB PT, INTERNATIONAL NORMALIZED RATIO	3100360	07/20/2026	Walther, Ryanne		1	Filed
HB CBC WITH PLT COUNT AND AUTO DIFF	3100386	07/20/2026	Walther, Ryanne		1	Filed
HB VENIPUNCTURE (NON ARUP)	3000002	07/20/2026	Walther, Ryanne		1	Filed
HB WILLOW REV 255	5040040	07/20/2026	Kump, Sean N	OPR	75	Filed
HB GLUCOSE BLOOD, REAGENT STRIP	3009669	07/20/2026	Kump, Sean N		1	Filed